Connecting the Dots

NHS NCS

Connecting the dots to restore independence, reduce waste and get people back to work.



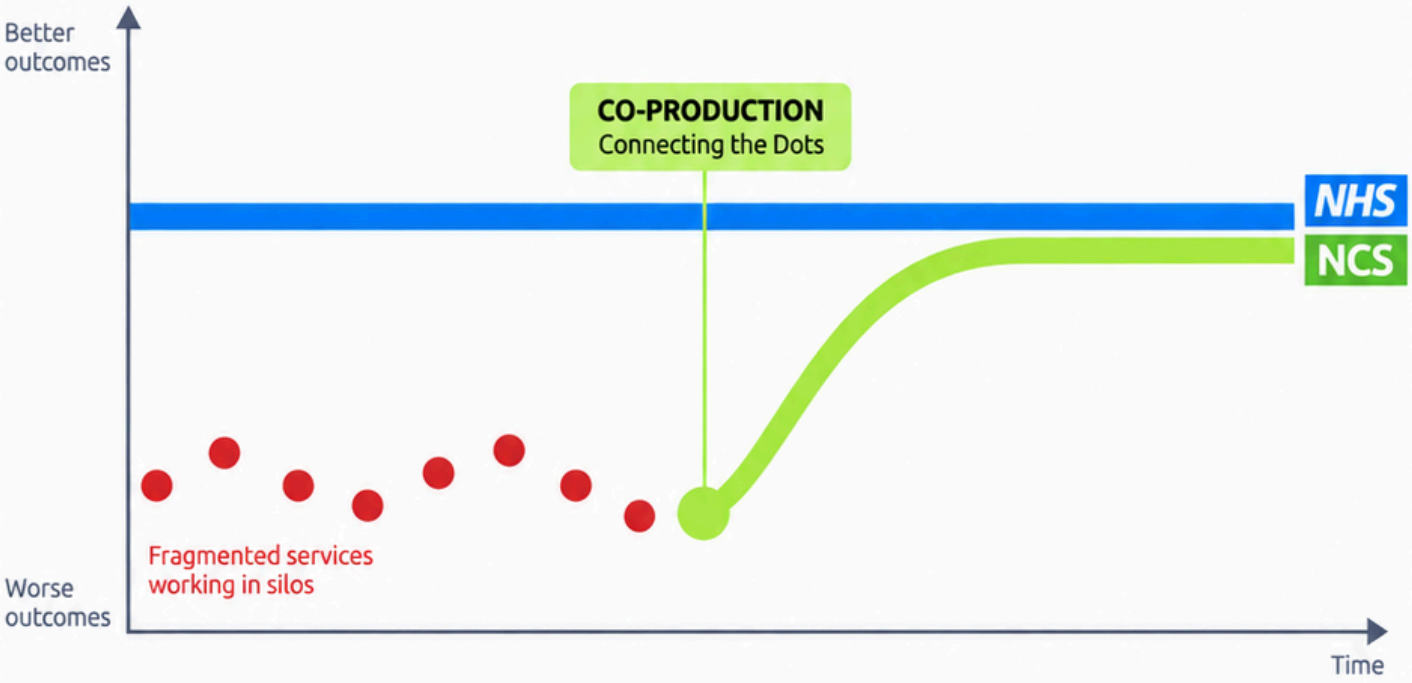
A practical NHS England test across five key regions.

20-family cohorts; scalable to 100 families.

Connecting the Dots

From fragmented care to better outcomes

People's health and care journey is currently split across disconnected organisations, systems and funding streams. Connecting the Dots creates a single, consent-based 360° view around each person and their carer, so the right people can work together, at the right time.



01

Fragmented today

Health and social care operate in silos, with no shared view of the person, their carer or their full support needs.

02

Connect the dots

A consent-based 360° household view links NHS, social care, housing, DWP, GPs and the voluntary sector.

03

Better outcomes

Earlier support, fewer crises, higher participation in learning, training and work, and more efficient use of public money.

03 | THE EVIDENCE

Six findings. Six national signals.

Six local findings, corroborated by research from several national sources.

66% **Joined-up services**

NHS service users reported at least one administration problem over 12 months.

Healthwatch / King's Fund et al., 2026

26.8% **Finding information**

Carers found information on support, services or benefits difficult to find.

DHSC Survey of Adult Carers, 2025–26

82.6% **Earlier carer support**

Carers providing 50+ hours weekly reported an adverse health effect.

ONS analysis (2015/17/19), 2024

31.4% **Continuity and inclusion**

Carers were unaware of discussions about support for the person they care for.

DHSC Survey of Adult Carers, 2025–26

30% **Accessible communication**

Respondents with communication needs rarely or never received needed support.

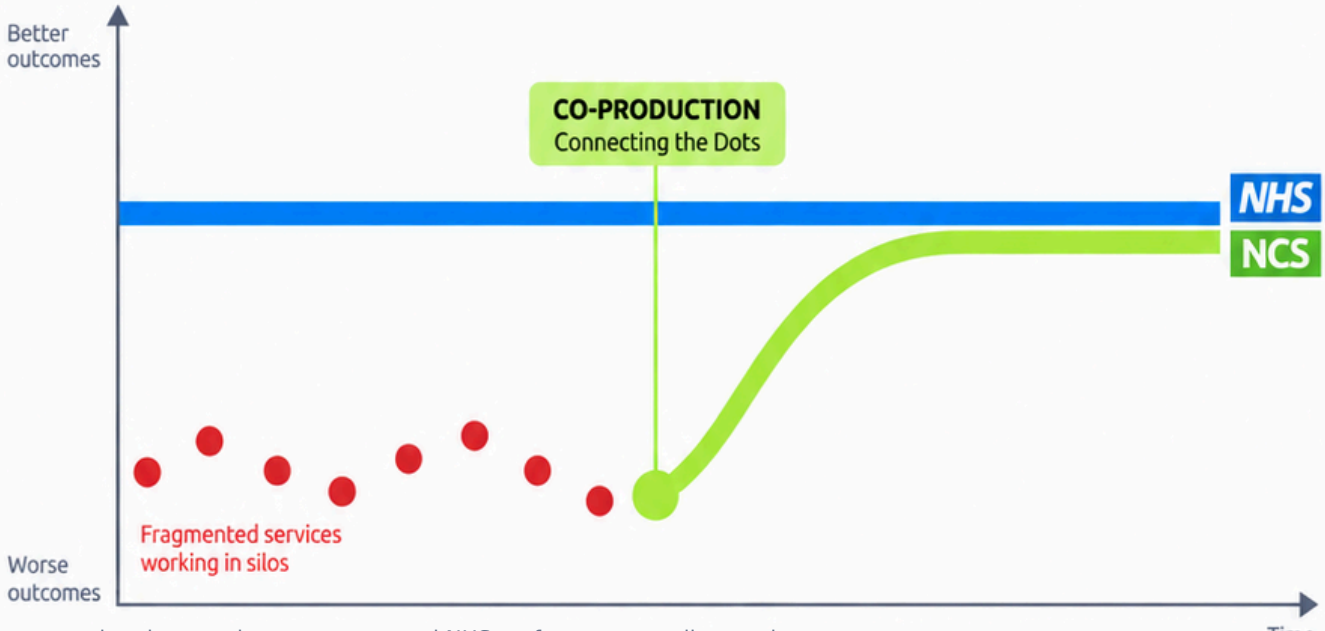
Healthwatch survey (self-selected), 2022

26.2% **Co-production and feedback**

Carers said they always felt involved or consulted in decisions.

DHSC Survey of Adult Carers, 2025–26

NHS REFERENCE / FRAGMENTED SUPPORT / CONNECTED CARE TEST

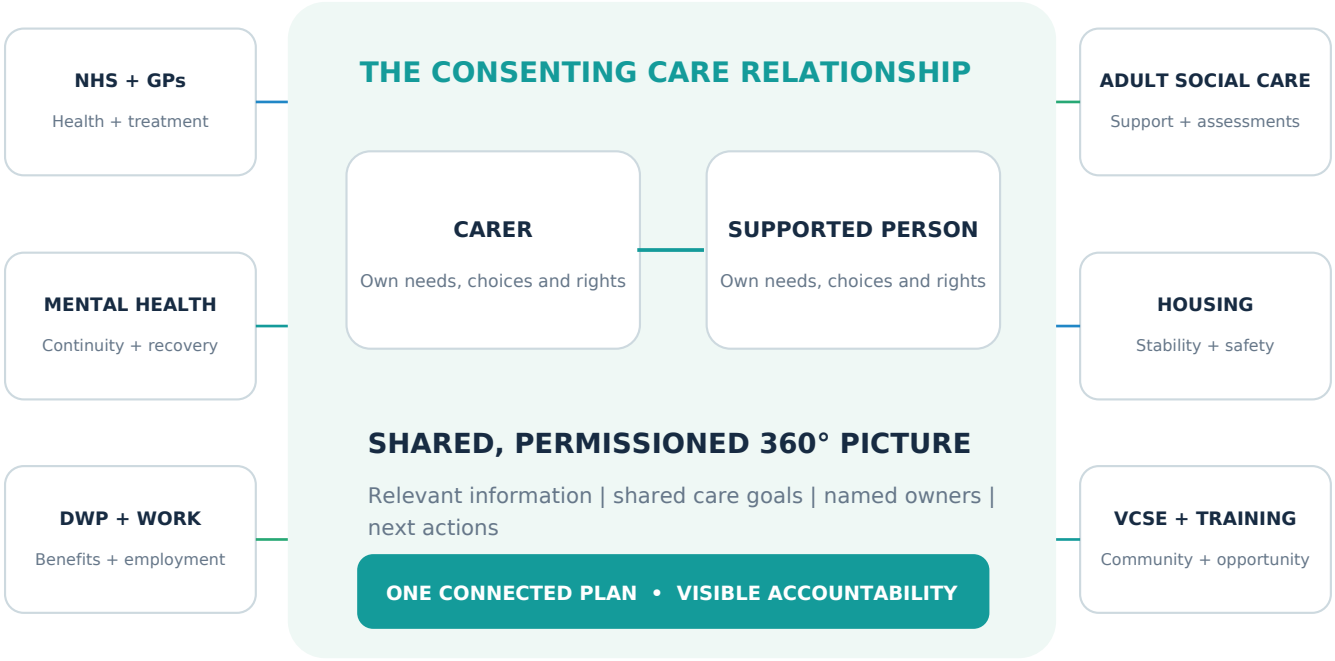


Conceptual pathway only — not measured NHS performance or pilot results.

Figures describe different populations and periods; see original evidence pages for definitions.

One household. One shared 360° view.

A shared, permissioned view of each care relationship: the household where relevant, never a merged identity.



ORGANISE THE PUBLIC SERVICES WE ALREADY HAVE AROUND ONE GOAL

SEE

Both voices and relevant facts

ACT

One lead and explicit next steps

MEASURE

Independence, time, work and cost

THE NATIONAL CARE SERVICE STARTS WITH A SHARED WAY OF WORKING

The switch is operational, not a claim that IT can be connected overnight: consent, lawful access, shared goals, named owners and measured outcomes. Each agency keeps its legal duties and existing systems.

1,500 people leave work to care each day.

Connecting the Dots would test whether coordinated support helps people retain or regain paid work - and restores opportunity for those they care for.

THE SCALE OF THE UK CHALLENGE

1,500 **people per day**

Estimated to have left paid work to provide unpaid care.

1.1m left employment in two years

6.7m estimated to combine work and unpaid care

TWO WAYS TO RESTORE ECONOMIC PARTICIPATION

PREVENT exits from work + HELP willing carers REGAIN paid hours

WHAT IF 2,000 PEOPLE REGAINED 8 PAID HOURS A WEEK?

An illustration of scale, not a forecast or a pilot target.

16,000

hours per week

2,000 people × 8 hours

427

full-time equivalents

at 37.5 hours per week

£9.98m

gross earnings / year

£13/hour × 48 weeks

THE 20-FAMILY TEST: MEASURE THE DIFFERENCE

Track paid hours retained or recovered, training participation, time spent coordinating care, costs of support, and independently evidenced changes in public expenditure.

Earnings are NOT tax receipts or cashable savings. Net fiscal effects must be evaluated.

Sources: Carers UK / Opinium, 16 Sept 2026; Carers UK Key facts and figures (2026).



A joined-up system for better lives

Connecting services and decisions can prevent problems, improve outcomes, and make better use of public resources.

TWO NATIONAL SERVICES. ONE CONNECTED LIFE.



National Health Service

Your health. Connected.

National Care Service

Your care. Connected.

The Government has committed to establishing a National Care Service. The Casey Commission has been asked to help determine how it should be implemented.

The opportunity is to build care as a connected national service - able to work alongside a connected health service.

CONNECT CARE.

RELEASE CAPACITY.

REDUCE COST.

IMPROVE OUTCOMES.

A National Care Service alongside the NHS

Two national services, working together around the same household.



WHAT CARE NEEDS TO CONNECT

Adult Social Care
Support, assessment and care services

Housing
Safe, suitable and stable homes

DWP & Benefits
Income, entitlements and financial support

Recovery & Rehabilitation
Treatment, therapy and long-term support

Mental Health
Integrated physical and mental health support

Learning & Training
Skills, confidence and personal development

Employment
Support into, in and back to work

Community Support
Voluntary, community and peer support

ONE HOUSEHOLD. TWO PEOPLE. ONE PLAN. A MORE CONNECTED LIFE.

Connect services and decisions around the household, with consent and clear responsibility.

What the evidence is built on

Carers, carees, people with lived experience and professionals contributed across a documented process.

11
meeting dates

9
official records

338
source-linked entries

41
named participants

16
directly attributed

6
recurring findings

WHAT PEOPLE DESCRIBED

Repeating information, chasing handovers and coordinating support across services while trying to care.

A documented pattern, gathered over three years of co-production.

The recurring burden is not a lack of effort by carers. It is the work of joining up support that arrives in separate pieces.

Six recurring findings

The practical changes repeatedly described across the Richmond and Wandsworth co-production record.

JOINED-UP SERVICES + SHARED RESPONSIBILITY

Make hand-offs and ownership visible across organisational boundaries.

ONE ROUTE TO ACCURATE INFORMATION + RECORDS

Create a reliable route to the right service so carers do not reconstruct the story.

EARLIER CARER SUPPORT

Identify carers early; plan assessment, respite and contingency.

PREVENTION + CONTINUITY BEFORE CRISIS

Keep support connected through discharge, accommodation and changes in responsibility.

ACCESSIBLE COMMUNICATION

Connect people to information while keeping a clear human route and alternatives.

CO-PRODUCTION THAT CLOSES THE LOOP

Show what changed because people contributed: action, owner, date and outcome.

PROVENANCE + APPROVAL ROUTE

Source basis: Richmond Borough and Wandsworth co-production records, reconciled across the documented meetings and follow-up material.
Provenance to be checked with Bruno Meekings and the working group before approval.

Findings offered for group confirmation; not unanimous end

04 | NATIONAL CORROBORATION

The same six issues appear in national evidence.

01 | JOINED-UP SERVICES + SHARED RESPONSIBILITY

49%

Of delayed discharges after acute stays of 14+ days were linked to social care capacity or transfer delays (23%), or access to rehabilitation, reablement or recovery (26%).

Source: CQC, The state of health care and adult social care in England 2024/25; NHS England data, March 2025.

02 | ONE ROUTE TO ACCURATE INFORMATION + RECORDS

26.8%

Of surveyed unpaid carers said it was fairly or very difficult to find information or advice about support, services or benefits.

Source: DHSC, Survey of Adult Carers in England 2025-26, information and advice.

03 | EARLIER CARER SUPPORT

20.9%

Of surveyed unpaid carers said they had no encouragement or support in their caring role.

Source: DHSC, Survey of Adult Carers in England 2025-26, support and services.

Scope: SACE measures carers' experience of support information; it does not measure shared records or repeated storytelling.

05 | NATIONAL CORROBORATION

The same six issues appear in national evidence.

04 | PREVENTION + CONTINUITY BEFORE CRISIS

335k

New-client requests for adult social care support in 2024-25 originated at hospital discharge: 17% of all such requests.

Source: DHSC, Adult social care activity report, England 2024-25, route of access.

05 | ACCESSIBLE COMMUNICATION

12.1%

Of adult social care service users who tried to find information or advice found it very difficult; 46,760 people in the survey.

Source: DHSC, Adult Social Care Survey 2024-25, knowledge and information.

06 | CO-PRODUCTION THAT CLOSES THE LOOP

38.1%

Of carers who knew discussions about the cared-for person’s services had happened said they were always involved or consulted.

Source: DHSC, Survey of Adult Carers in England 2025-26, involvement in arranging services.

Scope: these indicators corroborate the themes; they do not reproduce the local meeting sample or measure whether feedback changed services.

04 | NATIONAL CORROBORATION

Six local findings. Six national signals.

Three years of co-production independently identified the same terrain now visible in national evidence.

01 | JOINED-UP SERVICES

49%

Nearly half of delayed discharges were linked to social-care capacity, rehabilitation or recovery.

02 | ACCURATE INFORMATION

43.3%

Only 43.3% of carers found information about support, services or benefits very or fairly easy to find.

03 | EARLIER CARER SUPPORT

82.6%

Of people caring 50+ hours a week reported at least one adverse health effect, versus 43.0% at 1-19 hours.

04 | PREVENTION + CONTINUITY

58%

Of patients medically ready to leave hospital experienced a delayed discharge in March 2025.

05 | ACCESSIBLE COMMUNICATION

26.8%

Of carers found information and advice fairly or very difficult to find in the 2025-26 national survey.

06 | CO-PRODUCTION

26.2%

Only 26.2% said they always felt involved or consulted about support for the person they care for.

WHAT WE HEARD LOCALLY IS WHAT BRITAIN IS MEASURING NATIONALLY.

Sources: CQC State of Care 2024/25; DHSC Survey of Adult Carers in England 2025-26; ONS unpaid-care health analysis, published 2024.

We did not start with the national data.

WE LISTENED.

Three years of co-production independently identified the same problems now visible in national evidence.

CONNECTION	Services must join up around the household.
INFORMATION	Relevant facts should be visible to the right people.
EARLIER SUPPORT	Families need help before crisis becomes the norm.
CONTINUITY	Handoffs should preserve momentum, trust and progress.
COMMUNICATION	Updates must be timely, accessible and understandable.
PARTICIPATION	Carers and cared-for people must help shape decisions.

THE PROBLEM ISN'T LOCAL.
SO THE OPPORTUNITY SHOULDN'T BE EITHER.

THE NATIONAL SYSTEM CASE

Health and care: one system.

The NHS feels the cost of a broken handover.

REASONS FOR 14+ DAY DELAYED DISCHARGES | ENGLAND | MARCH 2025

49%

Nearly half linked to social care capacity, transfers, rehabilitation or recovery.

23% + 26% in CQC's analysis of NHS England data.

23%

Lack of social care capacity or delays completing transfers to social care.

26%

Delays accessing rehabilitation, reablement or recovery services.

CASEY'S REMIT RECOGNISES THE HEALTH–CARE INTERFACE

Its terms of reference call for a data-driven review of NHS and council funding at the health–care interface, with reforms to reduce unnecessary hospital admissions and delayed discharges. Connecting the Dots offers a small local test of shared oversight and named handovers: the same system question, measured in practice in Richmond and Wandsworth.

Sources: CQC, The state of health care and adult social care in England 2024/25, reporting NHS England discharge reasons for March 2025 (acute stays of 14+ days); Casey Commission terms of reference, GOV.UK, updated 1 October 2026.

Millions of people. One household at a time.

The scale is national, but improvement starts with a practical relationship and a measurable result.

5.8m

UNPAID CARERS | UK

Most recent comparable UK census total (2021). Many already work or are retired.

1.1m

LEFT PAID WORK TO CARE

Estimated number who left jobs to care in the two years before September 2026; polling, not a census.

889k

LONG-TERM CARE | ENGLAND

People receiving council-arranged long-term support during 2024-25; published figures subject to planned corrections.

16.7m

DISABLED PEOPLE | UK

2024-25 estimate. This is context, not a count of people needing care or suitable for paid work.

£29.4 BILLION

Gross local authority adult social care expenditure in England, 2024-25. This is existing spending - not a sum the pilot can simply redirect.

These populations overlap. They must not be added together or treated as a ready-to-work workforce.

Sources: Carers UK, DWP Family Resources Survey and DHSC 2024-25 (links in evidence note).

Before we connect the dots, count them.

The NHS has stronger internal pathways for its own services. The wider support system crosses organisations with different owners, duties and budgets.

INTEGRATED CARE SYSTEM (ICS)

A local partnership: NHS organisations, councils and other community partners plan together.

NHS SERVICE ARCHITECTURE

More connected internally — with seams between providers.

ICB | local NHS planning and commissioning

- **Primary care** — GP practices and PCNs
- **Hospital & specialist** — Trusts and specialist services
- **Community health** — Community teams and services
- **Mental health** — NHS mental health services

WIDER SUPPORT

Distinct owners and routes

- **COUNCIL** — Adult social care
- **HOUSING** — Council / housing providers
- **DWP** — Benefits and employment support

PARTNER BOUNDARY

ONE HOUSEHOLD | TWO PEOPLE: CARER + CAREE

Shared home, shared care: both carer and caree belong in the support picture.

TWO PEOPLE. ONE HOUSEHOLD.

Connor is one person; I am another. I was left without a social worker while continuing to care and pay, and lost our home and financial security. A system must see and support both of us.

CONNECTION IS ALSO A HUMAN EXPERIENCE

Aubrey's experience: "I leave Shine a Light / NHS sessions energised. Some borough meetings leave me needing 24 hours to recover."

A personal account of participation and meeting culture — not a survey finding.

Independent Commission on Adult Social Care
The Casey Commission

Evidence Submission
September 2026

Connecting the Dots

Remove the carer from the job of caring for a broken system.
Connect the decisions, not the databases.



4.7 million
unpaid carers in England (2021)

8.9%
of people aged 5+ provided unpaid care

2.4 million
provided 20+ hours of care

120,000
young carers aged 5-17

Better support gets people back to work.
A triple win for individuals, the economy and society.

Source: ONS Census 2021

Six recurring findings.

Across three years of co-production, contributors returned to six practical changes.

JOINED-UP SERVICES

Shared responsibility across the household’s support network.

ONE ROUTE TO INFORMATION

Consent-based support information for the household.

EARLIER CARER SUPPORT

Assessment, respite and contingency planning before crisis.

PREVENTION AND CONTINUITY

Support that acts early and stays connected through change.

ACCESSIBLE COMMUNICATION

Communication that includes both carer and caree.

CO-PRODUCTION

People and professionals connecting the dots together.

THE SHARED PROBLEM

Carers are still asked to connect services that do not connect themselves.

That costs carer time, professional capacity and public money — capacity that could support care, recovery, independence, learning and work.

Carers Connect.

One connected front door to health, care, home and work.



ONE HOUSEHOLD. TWO PEOPLE. ONE PLAN. A MORE CONNECTED LIFE.

A practical service concept: the right information, the right handover and the right support, in one everyday place.

Connecting the Dots.

A real-world demonstrator, designed with people in Richmond and Wandsworth.

20 CONSENTING HOUSEHOLDS

10 in Richmond and 10 in Wandsworth.

CONNECT THE RELEVANT SERVICES

A permissioned picture around each household, using the Co-production Charter principles.

NAME THE HANDOVERS

Make responsibility visible when a household’s support changes.

MEASURE BEFORE AND AFTER

Time, duplication, outcomes, carer and professional capacity, and progress towards independence.

START SMALL. LEARN QUICKLY. PROVE WHAT WORKS.

Six months is enough to test the method, learn with participants and show what should improve before any wider scale-up.

Start with achievable gains. Then tackle harder barriers.

All households receive support from the outset; the stages describe what outcomes we can test and learn from first.

01

EARLY GAINS

Keep or restore paid hours

Help willing carers resolve missed information, repeated assessments, coordination delays and barriers to flexible work.

MEASURE: Hours of paid work retained or regained; time saved.

02

NEXT OPPORTUNITIES

Learning, training and supported work

Coordinate rehabilitation, transport, benefits advice, employment support and accessible training where the person wants them.

MEASURE: New participation, confidence, skills and work readiness.

03

COMPLEX NEEDS

Stability before employability

For families facing multiple risks or high care needs, join decisions across health, housing and support; do not make employment a condition of care.

MEASURE: Continuity, avoided crises, independence and wellbeing.

20 CONSENTING HOUSEHOLDS. 10 + 10. SIX MONTHS.

Baseline first. Track change in every household. Record harms and non-improvements. Independent review.

Recovery first. Then measure the return.

A simple scenario, not a forecast: value restored work, then test whether government finances improve through tax, service efficiency, and better visibility of error and fraud.

ONE WILLING CARER REGAINS PART-TIME WORK

8 hours/week × £13/hour × 48 weeks

GROSS ANNUAL EARNINGS RESTORED **£4,992**

1 PERSON

£4,992

illustrative earnings / year

1,000 PEOPLE

£5.0m

illustrative earnings / year

58,000 PEOPLE

£289.5m

1% of the 5.8m carer census total

THE PUBLIC FINANCE TEST

ADDITIONAL TAX

Any verified change in tax receipts.

BENEFITS

Any verified change in entitlements or expenditure.

SERVICE USE

Avoided repeat work or crisis use where evidenced.

FRAUD + ERROR

Check whether shared relevant information surfaces duplicate payments, inconsistent records, ineligible claims or incorrect decisions.

COST TO DELIVER

Staff time, consent, data-sharing governance, systems and independent evaluation costs.

EARNINGS ≠ CASHABLE PUBLIC SAVINGS

Self-financing requires credible net fiscal gains to exceed added costs over time. Gross earnings are separate. Connecting the Dots may also help prevent fraud and administrative error by making inconsistencies visible sooner — a benefit the pilot should measure, not assume.

Help us test, measure and build the evidence.

This demonstrator is a practical way to test whether connecting care services around people improves the experience and use of public resources.

This proposition is offered in the context of the Government's commitment to a National Care Service and the work of the Independent Commission on Adult Social Care, chaired by Baroness Louise Casey of Blackstock DBE CB.

CONNECT services – **SAVE** time – **RELEASE** capacity – **IMPROVE** recovery – **SUPPORT** independence – **LEARN** what works

THE DEMONSTRATOR WILL TEST WHETHER IT CAN

- Reduce avoidable repetition, failed hand-offs and professional workload.
- Improve recovery, independence and participation for carers and carees.
- Release carers' capacity to care - and to work where possible.
- Use public money more effectively, with outcomes measured before claims are made.
- Produce a practical model that can inform national implementation.

EVIDENCE TO COLLECT

EXPERIENCE
What carers and carees say changes.

CAPACITY
Time saved or released across services.

OUTCOMES
Recovery, independence and participation.

We are not asking the Commission to accept the hypothesis.

We are asking the Commission to help us test it.

Pilot first. Platform second. Evidence before scale.

THE CASEY ASK

Enable the test. Convene the partners. Review the evidence.

A PROPOSED OVERSIGHT GROUP

CASEY COMMISSION	convene + independently review
NHS ENGLAND	digital, data + NHS App
SWL ICB	local NHS governance
ADULT SOCIAL CARE	Richmond + Wandsworth
HOUSING	council + provider partners
DWP + VCSE	benefits, work + community
CARERS + CAREES	co-design + consent
INDEPENDENT REVIEW	measure + publish results

WHAT WE ARE ASKING CASEY TO DO

1. Help convene the partnership and enable the six-month test.
2. Protect consent, confidentiality, separate rights and clear ownership.
3. Support an independent review and publication of the result.

We are not asking Casey to accept the hypothesis.
We are asking it to help us test it.

Pilot first. Platform second. Evidence before scale.

Sources, definitions and what remains to be tested.

The revised pages are a proposal for a measurable public-interest demonstrator - not a claim of proven returns.

UNPAID CARERS | 5.8m

Carers UK, Key facts and figures (most recent UK census data, 2021).

OPEN SOURCE / www.carersuk.org

WORK LOST TO CARE | 1.1m

Carers UK / Opinium, 16 Sep 2026; estimate of people leaving paid employment in the previous two years.

OPEN SOURCE / www.carersuk.org

DISABILITY | 16.7m

DWP Family Resources Survey, 2024-25; UK disability prevalence, not a measure of need for paid care.

OPEN SOURCE / www.gov.uk

LONG-TERM SOCIAL CARE | 889k

DHSC Activity Report, 2024-25: recipients during the year, not a concurrent caseload. DHSC flagged planned corrections on 7 Oct 2026.

OPEN SOURCE / www.gov.uk

SOCIAL CARE SPENDING | £29.4bn

DHSC Adult Social Care Finance Report, England, 2024-25; gross local authority current expenditure.

OPEN SOURCE / www.gov.uk

SCENARIO ASSUMPTIONS

8 paid hours/week × assumed £13/hour × 48 weeks. All earnings figures are gross and illustrative, not fiscal savings. The 1% applies only as arithmetic to the 5.8m carer benchmark.